d. Consider providing empiric therapy directed against MRSA in a patient with a prior history of MRSA infection; when the local prevalence** of MRSA colonization or infection is high; or if the infection is clinically severe.
IDSA 2012 guidelines (Adapted)
Explanatory notes:
* Risk factors for true infection with *Pseudomonas aeruginosa* include Immunocompromised status, Chronic Kidney Disease, warm climate and frequent exposure of foot to water.
**The local prevalence of MRSA (i.e., percentage of all *Staph. aureus* clinical isolates in that locale that are methicillin resistant) is high enough (perhaps 50% for a mild and 30% for a moderate soft tissue infection) that there is a reasonable probability of MRSA infection.

4.3.5e Give an initial antibiotic course for a soft tissue infection of about 1–2 weeks for mild infections and 2–3 weeks for moderate to severe infections. IDSA 2012 guidelines (Adapted)

4.3.5f Continue antibiotic therapy until, but not beyond, resolution of findings of infection, but not through complete healing of the wound. IDSA 2012 guidelines (Adapted)

4.3.5g Administer parenteral therapy initially for most severe infections and some moderate infections, with a switch to oral therapy when the infection is responding. (Strong; Low) IDSA 2012 guidelines (Adopted)

4.4: WOUND CARE
4.4.1 Clean ulcers regularly with clean water or saline*, debride them when possible in order to remove debris from the wound surface and dress them with a sterile, inert dressing in order to control excessive exudate and maintain a warm, moist environment in order to promote healing**. (Strong; Low)IWGDF 2015 (Adopted)
Explanatory note:
*Clean water is boiled cooled water (distilled water)
** Do not use hydrogen peroxide, EUSOL (Edinburgh University Solution), povidone iodine, chlorhexidine, etc

4.4.2 Select dressings principally on the basis of exudate control, comfort and cost. (Strong; Low) IWGDF 2015 (Adopted)

4.4.3 Do not use antimicrobial dressings with the goal of improving wound healing or preventing secondary infection. (Strong; Moderate) IWGDF 2015 (Adopted)

4.4.4 Do not offer the following to treat diabetic foot ulcers, unless as part of a clinical trial:
· Electrical stimulation therapy, autologous platelet-rich plasma gel, regenerative wound matrices and dalteparin.
· Growth factors (granulocyte colony-stimulating factor [G-CSF], platelet-derived growth factor [PDGF], epidermal growth factor [EGF] and transforming growth factor beta [TGF-β]).
· Hyperbaric oxygen therapy.
NICE clinical guideline NG19 (Adopted)

4.4.5 Consider dermal or skin substitutes as an adjunct to standard care when treating diabetic foot ulcers, only when healing has not progressed and on the advice of the multidisciplinary foot care service. NICE clinical guideline NG19 (Adopted)

4.4.6 1f Consider negative pressure wound therapy after surgical debridement for diabetic foot ulcers, on the advice of the multidisciplinary foot care service. NICE clinical guideline NG19 (Adopted)

4.4.7 Do not select agents reported to improve wound healing by altering the biology of the wound, including growth factors, bioengineered skin products and gases, in preference to accepted standards of good quality care. (Strong; Low) IWGDF 2015 (Adopted)

4.4.8 Do not select agents reported to have an impact on wound healing through alteration of the physical environment, including through the use of electricity, magnetism, ultrasound and shockwaves, in preference to accepted standards of good quality care. (Strong; Low) IWGDF 2015 (Adopted)

4.4.9 Do not select systemic treatments reported to improve wound healing, including drugs and herbal therapies, in preference to accepted standards of good quality care. (Strong; Low) IWGDF 2015 (Adopted)

4.4.10 Redistribution of pressure off the wound to the entire weight-bearing surface of the foot ("off-loading"). While particularly important for plantar wounds, this is also necessary to relieve pressure caused by dressings, footwear, or ambulation to any surface of the wound. (Strong; High) IDSA 2012 guidelines (Adopted)

4.4.11 When deciding about wound dressings and offloading when treating diabetic foot ulcers, take into account the clinical assessment of the wound and the person's preference, and use devices and dressings with the lowest acquisition cost appropriate to the clinical circumstances. NICE clinical guideline NG19 (Adopted)

4.5 : FOOTWEAR

4.5.1 To heal a neuropathic plantar forefoot ulcer without ischemia or uncontrolled infection in a patient with diabetes, offload with a non-removable knee-high device with an appropriate foot-device interface. (Strong; High) IWGDF 2015 (Adopted)

Non-removable (cast) walker: Same as removable (cast) boot/walker but then with a layer(s) of fibreglass cast material circumferentially wrapped around it rendering it irremovable (also known as "instant total contact cast")

4.5.2 When a non-removable knee-high device is contraindicated or not tolerated by the patient, consider offloading with a removable knee-high walker with an appropriate foot-device interface to heal a neuropathic plantar forefoot ulcer in a patient with diabetes, but only when the patient can be expected to be adherent to wearing the device. (Weak; Moderate) IWGDF 2015 (Adopted)

Removable (cast) boot/walker: Prefabricated removable knee-high boot with a rocker or roller outsole configuration, padded interior, and an insertable and adjustable insole which may be total contact.

4.5.3 When a knee-high device is contraindicated or cannot be tolerated by the patient, consider offloading with a forefoot offloading shoe, cast shoe, or custom-made temporary shoe to heal a neuropathic plantar forefoot ulcer in a patient with diabetes,